



File No.: EXP202308486

RESOLUTION ON THE ARCHIVE OF PROCEEDINGS

From the actions taken by the Spanish Agency for Data Protection and based on the following

FACTS

FIRST: On May 31, 2023, a complaint was filed with the Spanish Agency for Data Protection (AEPD) against the RIOJAN HEALTH SERVICE with NIF Q2669003B (hereinafter, the respondent party or HEALTH SERVICE). The reasons for the complaint are as follows:

In the received document, the complainant states the following:

The complainant explains that on two occasions their personal data has been confused with that of third parties who have the same name and surname but different DNI numbers. They allege that this occurred in 2020 with a man from Pontevedra, which resulted in them being deregistered from Social Security, losing the right to public health care, and their medical history being merged with that of the citizen from Pontevedra. This situation was reversed after numerous efforts.

Furthermore, they state that in 2023 it happened again, this time with a man from Arnedo, La Rioja, being aware of what had occurred during a medical consultation, where their medical history had disappeared again.

They provide a copy of: "Document for the issuance and/or modification of the health card," dated 07/14/2020, "Document certifying the right to health care," dated 07/09/2020, "Resolution: Assignment of Social Security number or affiliate number," dated 06/15/2020.

They mention that they have been informed that it is impossible to recover their medical history. They do not provide documentation supporting the account regarding the year 2023.

SECOND: In accordance with Article 65.4 of Organic Law 3/2018, of December 5, on Personal Data Protection and Guarantee of Digital Rights (hereinafter LOPDGDD), this complaint was forwarded to the respondent party for analysis and to inform this Agency within one month of the actions taken to comply with the requirements set forth in data protection regulations.

Although the notification was validly carried out by electronic means, and the procedure was considered completed in accordance with the provisions of Article 41.5 of the LPACAP, a copy was sent by postal mail for informational purposes, which was duly notified on 07/19/2023. In this notification, they were reminded of their obligation to communicate electronically with the Administration, and were informed of the means of access to such notifications, reiterating that, henceforth, notifications would be made exclusively by electronic means.

C/ Jorge Juan, 6

www.aepd.es

28001 – Madrid

sedeagpd.gob.es

No response was received to this forwarding letter within the granted period, although a response was received after the initiation of the preliminary investigation proceedings detailed in the following sections of this resolution.

THIRD: On August 31, 2023, in accordance with Article 65 of the LOPDGDD, the complaint was admitted for processing.

FOURTH: The General Sub-Directorate for Data Inspection proceeded to carry out preliminary investigation actions to clarify the facts in question, by virtue of the functions assigned to control authorities in Article 57.1 and the powers granted in Article 58.1 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and in accordance with the provisions of Title VII, Chapter I, Section two, of the LOPDGDD, having knowledge of the following aspects:

- On 10/20/2023, a written response to the forwarding of the complaint was received and incorporated into these proceedings. In the mentioned document, the HEALTH SERVICE states:

- o In the checks carried out, the existence of various persons with the same name and two surnames

as the complainant with different DNI numbers has been detected. Furthermore, it has been verified that the complainant's medical history has not disappeared.

- o On September 18 and 19, 2023, attempts were made to contact the complainant without success. On September 26, 2023, contact was made with them by telephone, where they were informed that it was a punctual error and the measures taken to prevent it from happening again, and on October 4, 2023, this information was sent to the complainant in writing.

- o The HEALTH SERVICE has provided proof of the shipment, which includes an envelope sent to the complainant's address without its contents.

- On 11/17/2023, the inspection sent a request to the HEALTH SERVICE to know the description of the specific events that occurred in 2020 and 2023 regarding the complainant's medical history, as well as the specification of the procedures for the recovery of the complainant's medical history, reiterated on 02/28/2024.

On 03/19/2024, a document from the representative of the respondent party was received with the same response as that given to the forwarding letter, submitted on 10/20/2023, although this time it includes the document sent to the complainant on 10/04/2023, informing them of the following:

"The errors occurred when searching in our Electronic Medical History database for your personal history through name and surnames, attributing it to a patient with the same name, but who was not you. This means that you did not temporarily appear as a user of SERIS; but without loss of information, which continued in your inactive medical history. At this time, the situation has been reviewed, improving the search for patients through the CIPA (Autonomous Identification Card Number),

C/ Jorge Juan, 6

www.aepd.es

28001 – Madrid

sedeagpd.gob.es

3/14

avoiding the creation of new patients without first ensuring that they do not exist in our user database and verifying their affiliation or not to the Health Card.

Once their status of registration in the SERIS has been recovered, it has been verified both in the specialized Clinical History and in the primary care Clinical History that their data is present and correct according to their information."

-

On the date of 25/4/2024, information was requested from the HEALTH SERVICE by the General Subdirectorate of Data Inspection regarding the measures adopted to prevent similar incidents from occurring, implementation dates, and controls carried out to verify their effectiveness, as well as the submission of technical reports or recommendations prepared by the Data Protection Delegate or by the security officer, regardless of their format, concerning the treatments for which information is requested, and about subsequent actions taken or their absence, derived from the aforementioned technical reports or recommendations. On the date of 02/05/2024, a response letter was received in which the representative of the responding party states the following:

o

The Riojan Health Service had already begun these improvement works in the search for patients in the applications related to the clinical history, using a unique identifier. The proposed and accepted decision by the responsible party was to use the Autonomous Personal Identification Code (CIPA) as the unique identification data. Regarding this initiative, and as evidence, they provide the Quality and Safety Coordination Report 2022-2023 of the Riojan Health Service (SERIS), which includes the point "Quality and Safety Project: Implementation of the use of CIPA as a unique patient identifier in SERIS."

o

Once this improvement was implemented, its effectiveness was verified and the information and working method were disseminated throughout the organization and, especially, to the professionals involved in the reception of patients in the different healthcare areas.

o

Among other initiatives, the Quality and Safety Coordination Department carried out widespread

dissemination through informative documents that were published and distributed in the organization's Newsletter, which is sent by email to the entire staff. They provide screenshots of the articles from the Newsletter dated October 11, 2022, and February 9, 2023.

o

From the technology area, utilities and tools were developed that allow for the analysis and monitoring of histories not linked to the health card, as well as duplications or errors in the creation of new histories. These utilities facilitate the early detection of possible error cases and their correction (deletion or merging of clinical histories) by the staff of the Archive and Clinical Documentation Service.

o

Once the Data Protection Delegate, who is a collegiate body composed of a multidisciplinary team of people from the

C/ Jorge Juan, 6

www.aepd.es

28001 – Madrid

sedeagpd.gob.es

4/14

organization, was aware of the incident, and proceeded to gather all the information regarding it, especially about the measures adopted to improve the unique identification process of patients. In this line of collaboration with the Data Controller, a member of that collegiate body (DPO), specifically the Head of the Archive and Clinical Documentation Service, joined the working team that led all the initiatives and improvement plans detailed in the previous point. Thus, on one hand, the basic security requirements to avoid risks of identification problems proposed by the DPO were successfully conveyed, and at the same time, the DPO was kept permanently informed and updated on the progress of those initiatives.

In the introduction of the Coordination Report on Quality and Patient Safety 2022-2023 of the Riojan Health Service (SERIS) provided, it is specified that the Coordination of Quality and Patient Safety of the claimed party began its activity on May 1, 2022, achieving the performance of its work in close collaboration with the Strategic Planning Directorate, from which it depends. In the first phase, the analysis and collection of existing information and documentation in the different services and healthcare centers of the organization were carried out. At the same time, information was provided about its recent constitution and commencement of work with all of SERIS, the professional in charge of its direction, the Patient Safety (PS) policy promoted by the Management, and the ongoing project for the development and implementation of the PSP-SERIS. This work was achieved through a round of 48 face-to-face meetings, personally conducted by the coordinator and the director, with executives and middle management, as well as with different healthcare professionals currently linked or who were in the past, with Patient Safety (PS) activities in the San Pedro University Hospital, HUSP, HC, HGLR, Primary Care, and Emergency Care (061).

Point 3 of the report outlines the planned and fulfilled objectives: preparation and publication of the SERIS Patient Safety Plan 2022-2024, and design and consolidation of the appropriate organizational and functional structure to carry out the defined strategic lines of Patient Safety of SERIS.

-

On the dates of 9/5/2024 and 21/6/2024, the General Subdirectorate of Data Inspection reiterated to the claimed party the information requested on 17/11/2023, reiterated on 28/2/2024, in order to know the description of the specific facts that occurred in 2020 and 2023 regarding the claimant's medical history, as well as the specification of the procedures for the recovery of the claimant's medical history. On 18/7/2024, a written response to the request was received in which they state the following:

o

The communications between the population database of La Rioja (Health Card), the SELENE application of digital medical history in La Rioja, and the Health Card of the National Health System, do not require the exchange of documentation, but are carried out through electronic messaging. They provide a copy of the emails that have been generated collecting the necessary information.

C/ Jorge Juan, 6

www.aepd.es
28001 – Madrid
sedeagpd.gob.es
5/14

o

The facts that led to the issue referred to by the user are as follows:

- Both the patient from La Rioja and the patient from Pontevedra share the same name, surname, and date of birth.
- In April 2016, upon turning 14 years old, the DNI and NAF (Affiliation Number) were added to the health card data of the patient from La Rioja. In this regard, they provide a screenshot of the software application that collects this data.
- In May 2017, the National Health System (SNS) made a modification of data based on the daily movements of the National Institute of Social Security (INSS), which erroneously and automatically linked the NAF and DNI data of the user from La Rioja to those of the user from Galicia. In this regard, they provide a screenshot of the software application.
- In January 2019, the health card service of La Rioja received an email from the Health Card Service of the Ministry, indicating that they found a user (A.A.A.) who appears with two SNS codes (**CODE.1 AND **CODE.2) and communicated that they have proceeded to deactivate the SNS code **CODE.2 due to duplication, which was linked to the Autonomous Personal Identification Code (CIP-AUT) of the user from La Rioja, leaving him without the right to health care and his Electronic Clinical History (HCE) inactive. In this regard, a screenshot of that email is provided.
- The Health Card Service requests information regarding this matter and is informed that the previous action by the Health Card Service of the Ministry was carried out at the request of Galicia. A screenshot of that communication is provided.
- In 2020, when the patient from La Rioja went to the La Rioja Health Service requesting assistance, the professional who attended him, with the search data at that moment (a search that has already been improved to achieve the unique identification of the patient, as indicated in previous writings addressed to the AEPD), only found the patient from Pontevedra. Hence the user's feeling that their clinical history has been usurped, which does not correspond to reality as it was only inactive and does not require any communication with the Galician Health Service, which only manages the clinical history of the Galician patient.

o

The recovery of the visibility of the claimant's clinical history only involved reactivating him in the Health Card system with his correct data and assigning him a new Autonomous Personal Identification Code (CIP-AUT), which is the link to the Electronic Clinical History (HCE) application. This rescued his clinical history from the repository (database of previously treated patients without an active health card) and it became an active history with all his rights and without loss of information.

C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
sedeagpd.gob.es

6/14

The claimant's clinical history was not associated with a third person but was inactive from 23.01.19 to 14.07.20.

o

Regarding the events that occurred in 2023, after conversations held with the claimant and his mother (who was the person attending the health centers requesting the service for her son), they do not exactly correspond with what the patient describes. The clinical history was active, belonged to the claimant patient from La Rioja, but the medical professional, when prescribing a prescription, must have encountered some connectivity issue between the clinical history and the electronic prescription, and we deduce from his writing that the claimant experienced it as if the problem from 2020 was repeating itself, although the situation was not the same in any case. The patient from Arnedo

mentioned appeared as a result of the search carried out by the healthcare professional, with whom our claimant only shares name and surname, ending the coincidences there.

C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
sedeagpd.gob.es
7/14

Therefore, in the preliminary investigation proceedings carried out by the AEPD, the following conclusions are reached:

- The complainant states that, on two occasions, in 2020 and in 2023, their personal data has been confused with that of third parties who share the same name and surname but have different ID numbers. The second time, they became aware of what had happened during a medical consultation, having found that their medical history had disappeared again. They report that they have been informed that it is impossible to recover their medical history. No supporting documentation of the claims has been provided.
- From the documentation and information provided by the respondent, it is evident that the events that led to the issue reported by the complainant are as follows:
 - o Both the complainant and the patient from Pontevedra share the same name, surname, and date of birth.
 - o In April 2016, upon turning 14 years old, the ID number and NAF were added to the complainant's health card data.
 - o In May 2017, the National Health System (SNS) made a modification of data based on the daily movements of the INSS, which erroneously and automatically linked the complainant's NAF and ID number, assigning them to the corresponding Galician user.
 - o In January 2019, the health card service of La Rioja received an email from the Health Card Service of the Ministry, indicating that they had found a user (A.A.A.) (**CODE.1 AND **CODE.2) and communicated that they had proceeded to deactivate the SNS code **CODE.2 due to duplication, which was linked to the Autonomous Personal Identification Code (CIP-AUT) of the complainant, leaving them without the right to health care and their electronic medical history inactive.
 - o When searching in the electronic medical history database for their personal history through name and surname, it was linked to a patient with the same name, but it was not them. This meant that they did not temporarily appear as a user of the HEALTH SERVICE; however, there was no loss of information, which continued to exist in their inactive medical history. The events occurred due to the existing protocol at that time for accessing patients' medical histories, as the search was conducted using the patient's name and surname, which could lead to confusion between patients with the same data. Therefore, the HEALTH SERVICE has adopted measures to prevent such events from recurring.

C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
sedeagpd.gob.es

8/14

- o The complainant's medical history was never associated with a third party but remained inactive from January 23, 2019, to July 14, 2020.
- o Regarding the events that occurred in 2023, the medical history was active, belonged to the complainant, but the medical professional, when prescribing a prescription, must have encountered some connectivity issue between the medical history and the electronic prescription. They assert that, although the complainant may have experienced it as if the problem from 2020 was repeating, the situation was not the same in any case.

FIFTH: On July 29, 2024, within the framework of the investigation proceedings referred to in the previous factual background, the General Subdirectorate of Data Inspection requested information and documentation from the Ministry of Health regarding the events that occurred in May 2017, for which the National Health System (SNS) made a modification of data based on the daily movements of the

INSS, which erroneously and automatically linked the complainant's NAF and ID number, assigning them to the corresponding Galician user.

On October 21, 2024, a response was received from the Ministry of Health, which broadly explains the functioning of the databases referenced in the responses by the respondent. Additionally, the following information regarding the specific case is provided:

- Two successive movements occur from the Health Insurance Database of the INSS (BADAS): on May 10, 2017, an ID number and a Social Security affiliation number (NAF) are assigned, and on June 22, 2017, the NAF of another person, with the same name and surname as a user who had been registered in Galicia until that moment, is assigned.
- Following this event, as there were two people in the Protected Population Database (BDPPSNS) with the same ID number, NAF, name, and surname, the personal identification code of the National Health System that the complainant had was deactivated due to the duplication it was causing in the database. Subsequently, SERIS assigned them another personal identification code to reverse the situation, which was registered as active on July 15, 2020, resolving the situation.
- As of October 18, 2024, the situation of A.A.A. with ID number ***NIF.1 is correct. The current (...) is ***CODE.3, ID ***NIF.1, (...) ***CODE.4, correct name and surname, A.A.A. (...).
- Therefore, according to what is recorded in the Protected Population Database (BDPPSNS), the situation does not revert to the data of another person at any time since the activation on July 15, 2020.

LEGAL GROUNDS

I

Competence

C/ Jorge Juan, 6

www.aepd.es

28001 – Madrid

sedeagpd.gob.es

9/14

In accordance with the functions conferred upon each supervisory authority by Article 57.1 a), f), and h) of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR) and as provided in Articles 47 and 48.1 of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the Director of the Spanish Agency for Data Protection is competent to resolve these investigative actions.

Furthermore, Article 63.2 of the LOPDGDD states: "The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of Regulation (EU) 2016/679, this organic law, the regulatory provisions issued for its development, and, in so far as they do not contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In the present case, in accordance with the provisions of Article 4.1 of the GDPR, it is established that there is a processing of personal data, since the HEALTH SERVICE carries out, among other processes, the collection, storage, and use of personal data of individuals.

The party being claimed carries out this activity in its capacity as the data controller, as it is the one who determines the purposes and means of such activity, by virtue of the aforementioned Article 4.7 of the GDPR.

III

Security of Processing

Article 32 "Security of Processing" of the GDPR establishes:

"1. Taking into account the state of the art, the costs of implementation, and the nature, scope, context, and purposes of the processing, as well as the risks of varying probability and severity for the rights and freedoms of natural persons, the controller and the processor shall implement appropriate technical and organizational measures to ensure a level of security appropriate to the risk, which may include, among others:

- a) the pseudonymization and encryption of personal data;
 - b) the ability to ensure the ongoing confidentiality, integrity, availability, and resilience of processing systems and services;
 - c) the ability to restore the availability and access to personal data quickly in the event of a physical or technical incident;
 - d) a process for regularly verifying, assessing, and evaluating the effectiveness of the technical and organizational measures to ensure the security of processing.
2. In assessing the appropriateness of the level of security, particular consideration shall be given to the risks presented by the processing of data, in particular as a consequence of the accidental or unlawful destruction, loss, or alteration of personal data transmitted, stored, or otherwise processed, or the unauthorized communication or access to such data.
3. Adherence to an approved code of conduct pursuant to Article 40 or to an approved certification mechanism pursuant to Article 42 may serve as an element to demonstrate compliance with the requirements set out in paragraph 1 of this article.
4. The controller and the processor shall take measures to ensure that any person acting under the authority of the controller or the processor and having access to personal data may only process such data on instructions from the controller, unless required to do so by Union or Member State law.”

IV

Data Protection by Design and by Default

Article 25 of the GDPR establishes the following:

- 1. Taking into account the state of the art, the cost of implementation, and the nature, scope, context, and purposes of the processing, as well as the risks of varying probability and severity that the processing entails for the rights and freedoms of natural persons, the controller shall implement, both at the time of determining the means of processing and at the time of the processing itself, appropriate technical and organizational measures, such as pseudonymization, designed to effectively apply the principles of data protection, such as data minimization, and integrate the necessary safeguards into the processing, in order to comply with the requirements of this Regulation and protect the rights of data subjects.
- 2. The controller shall implement appropriate technical and organizational measures to ensure that, by default, only personal data that are necessary for each specific purpose of the processing are processed. This obligation shall apply to the amount of personal data collected, the extent of their processing, the period of their storage, and their accessibility. Such measures shall ensure in particular that, by default, personal data are not accessible, without the intervention of the individual, to an indefinite number of natural persons.
- 3. An approved certification mechanism pursuant to Article 42 may be used as an element to demonstrate compliance with the obligations set out in paragraphs 1 and 2 of this article.

VII

C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
sedeagpd.gob.es

11/14

Conclusion

From the documentation in this procedure, the following is deduced:

C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
sedeagpd.gob.es

12/14

There are two differentiated situations that cannot be classified as equivalent from the perspective of data protection.

-
The first of these, identified by the claimant as occurring in 2020, involved an erroneous linkage made in 2017 of the claimant's NAF and DNI data with those corresponding to a third party with whom they shared name, surname, and date of birth. As a result of this duplication, their Personal Identification Code of the National Health System was deactivated in January 2019, which also caused their clinical history to be inactive from January 23, 2019, until July 14, 2020. Consequently, when searching for the claimant's clinical history using the parameters established in the corresponding protocol (name and surname), the clinical history of the third party with whom they shared name, surname, and date of birth appeared. This specific situation was resolved by the HEALTH SERVICE on July 15, 2020, by assigning a new Autonomous Personal Identification Code (CIPA).
-

On the other hand, the second situation, which occurred in 2023, prevented the claimant from being found in the HEALTH SERVICE database on a specific occasion, without any case of deactivation due to duplication in the system, their data being linked to any other user, or their clinical history being inactive. Thus, the Protected Population Database (BDPPSNS) does not show that any changes have occurred regarding the claimant since July 15, 2020, the date on which the respondent assigned them another Autonomous Personal Identification Code (CIPA), and it correctly records the claimant's data (name, surname, DNI, CIPSNS, CIPA, and NAF). In light of the above, as indicated, both situations are not comparable, and only the first would give rise to possible infringements in data protection matters. Regarding the actions taken by the respondent following the events that occurred in the first of the cases, which could indeed constitute an infringement, it is noteworthy that the respondent decided to use the Autonomous Personal Identification Code (CIPA) as a unique identification data point, which helps avoid errors in patient searches using the parameters of name and surname, as well as verifying their inclusion in the different databases.

C/ Jorge Juan, 6
www.aepd.es
28001 – Madrid
sedeagpd.gob.es

13/14

In this regard, the Department of Quality and Safety Coordination carried out widespread dissemination through informational documents that were published and distributed in the organization's Newsletter, which is sent via email to the entire staff. They provide screenshots of the articles from the Newsletter dated October 11, 2022, and February 9, 2023. This demonstrates the adoption of measures to prevent situations like this at least since October 2022.

Any possible infringements that may have been committed in relation to these events would have a statute of limitations, in accordance with Article 73 of the LOPDGDD, of 2 years:

“According to what is established in Article 83.4 of Regulation (EU) 2016/679, the following infringements are considered serious and will be subject to a two-year statute of limitations for substantial violations of the articles mentioned therein, particularly the following:

“f) The failure to adopt those technical and organizational measures that are appropriate to ensure a level of security adequate to the risk of processing, as required by Article 32.1 of Regulation (EU) 2016/679.

d) The failure to adopt those technical and organizational measures that are appropriate to effectively apply the principles of data protection by design, as well as the failure to integrate the necessary guarantees in processing, as required by Article 25 of Regulation (EU) 2016/679.”

Therefore, based on the information provided in the previous paragraphs, any possible infringements that may have been committed would be time-barred.

Thus, in accordance with the aforementioned, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO ARCHIVE the present proceedings.

SECOND: TO NOTIFY this resolution to the RIOJAN HEALTH SERVICE and to the claimant.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public

once it has been notified to the interested parties.

Against this resolution, which concludes the administrative process as prescribed by Article 114.1.c) of Law 39/2015, of October 1, on the Common Administrative Procedure of Public Administrations, and in accordance with the provisions of Articles 112 and 123 of the aforementioned Law 39/2015, of October 1, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file a contentious-administrative appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and Section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Jurisdiction

C/ Jorge Juan, 6

www.aepd.es

28001 – Madrid

sedeagpd.gob.es

14/14

Contentious-Administrative, within a period of two months from the day following the notification of this act, as provided for in Article 46.1 of the aforementioned Law.

1245-21112023

Mar España Martí

Director of the Spanish Agency for Data Protection

C/ Jorge Juan, 6

www.aepd.es

28001 – Madrid

sedeagpd.gob.es